

Pennsylvania 2027 Health Insurance Rate Filings and Open Enrollment Timeline

Prepared from the Pennsylvania Insurance Department's July 23, 2026 announcement and the supplied Pennie, CMS, and litigation records. Requested rates are not final approved premiums.

Executive summary

Pennsylvania insurers requested an enrollment-weighted average premium increase of **17.1% in the individual market** and **11.5% in the small-group market** for Plan Year 2027. The Pennsylvania Insurance Department stated that final rates would be published in the fall after review of the filings, objections, and public comments (Pennsylvania Insurance Department [PID], 2026). <citation src="2"></citation>

The largest individual-market request was **Ambetter Health of Pennsylvania at 40.90%**. PID explained that this figure combines a **23.44% silver-to-silver increase** with a **17.46% bronze-to-silver mapping differential** resulting from Ambetter's planned withdrawal from the bronze tier. Consumers losing Ambetter bronze plans were expected to have access to bronze plans offered by other carriers (PID, 2026). <citation src="2"></citation>

The 2027 open-enrollment schedule also changed. Pennie initially moved toward a shorter October 15–December 15 enrollment period, but litigation over the federal nine-week limit resulted in restoration of the traditional **November 1, 2026–January 15, 2027** schedule (Pennsylvania Office of Rural Health [PORH], 2026). <citation src="5"></citation>

2027 individual-market rate requests

The individual-market requests supplied in the record are listed below. The percentages are **average requested rate changes**, not necessarily the change a particular consumer will experience.

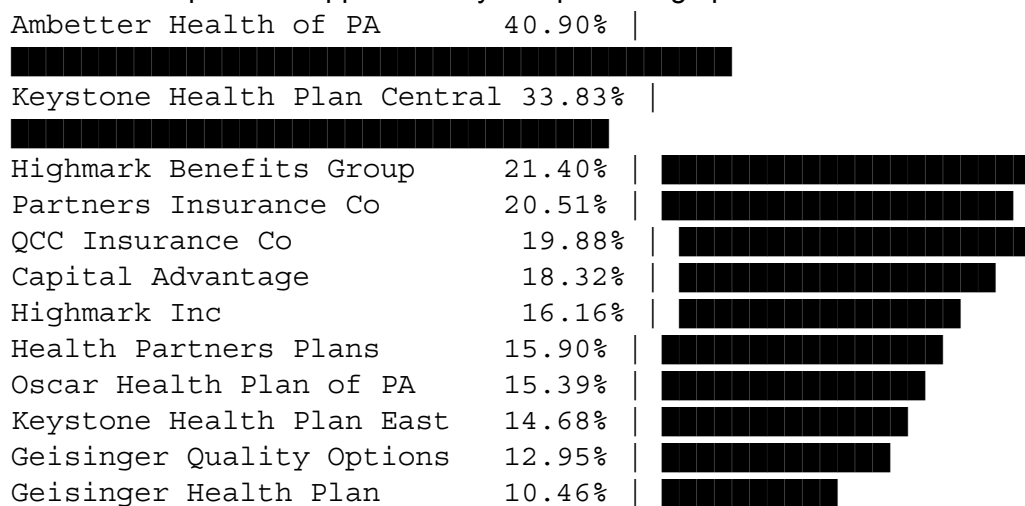
Carrier	Requested increase	Rating areas
Ambetter Health of Pennsylvania	40.90%	1–9
Capital Advantage Assurance Company	18.32%	6, 7, 9
Geisinger Health Plan	10.46%	2, 3, 5, 6, 7, 9
Geisinger Quality Options	12.95%	2, 3, 5, 6, 7, 9
Health Partners Plans	15.90%	3, 6, 8
Highmark Benefits Group	21.40%	3, 8
Highmark Inc.	16.16%	1, 2, 4, 5, 6, 7, 9
Keystone Health Plan Central	33.83%	6, 7, 9
Keystone Health Plan East	14.68%	8
Oscar Health Plan of	15.39%	3, 6, 7, 9

Carrier	Requested increase	Rating areas
Pennsylvania		
Partners Insurance Company	20.51%	3, 6, 8
QCC Insurance Company	19.88%	8

Additional individual-market filings identified in PID's primary announcement: UPMC Health Network, Inc. requested 12.03%, and UPMC Health Plan, Inc. requested 15.82%. These two filings were not included in the supplied carrier list (PID, 2026). <citation src="2"></citation>

Individual-market graph

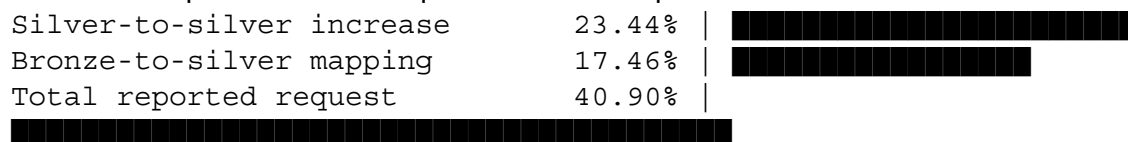
Each block represents approximately one percentage point.



The difference between the highest and lowest supplied individual-market requests is **30.44 percentage points**. Ambetter's request is approximately **3.9 times** Geisinger Health Plan's request, although the two carriers may serve different regions and plan populations.

Ambetter's plan-mapping effect

Ambetter's reported 40.90% request has two components:



The second component reflects the transition of affected bronze enrollees into silver plans after Ambetter's planned exit from the bronze metal tier. Therefore, the 40.90% figure should not be interpreted as a uniform premium increase for every current Ambetter member.

2027 small-group market rate requests

The supplied record includes the following small-group filings:

Carrier	Requested increase	Rating areas or notes
Geisinger Quality Options	4.05%	2, 3, 5, 6, 7, 9

Carrier	Requested increase	Rating areas or notes
Highmark Benefits Group	16.99%	6, 7, 9
Highmark Care Benefits	19.95%	3
Highmark Coverage Advantage	14.63%	1, 2, 4, 5, 6
Highmark Inc.	15.57%	1, 2, 4, 5, 6, 7, 9
Highmark Senior Health Company	14.32%	8
UnitedHealthcare of Pennsylvania	Not stated	Includes annual adjustments and Gold/Silver-to-Bronze plan mapping

PID's primary announcement also identified additional small-group filings omitted from the supplied list, including Capital Advantage Assurance Company at 16.11%, Geisinger Health Plan at 3.74%, Independence Assurance Company at 8.95%, and Keystone Health Plan Central at 20.93% (PID, 2026). <citation src="2"></citation>

Small-group graph

Highmark Care Benefits	19.95%	
Highmark Benefits Group	16.99%	
Highmark Inc	15.57%	
Highmark Coverage Advantage	14.63%	
Highmark Senior Health Co.	14.32%	
Geisinger Quality Options	4.05%	

The supplied small-group requests range from **4.05% to 19.95%**, a spread of **15.90 percentage points**. The enrollment-weighted small-group average, however, is **11.5%**, so the average cannot be reproduced by simply averaging the carrier percentages (PID, 2026). <citation src="2"></citation>

Enrollment attrition

The supplied Pennie series reports the following active-enrollment levels:

Feb. 1	486,000	
Mar. 24	471,442	
May 1	452,000	
July 1	431,270	
Aug. 1	422,869	

From February 1 to August 1, the reported active enrollment declined by **63,131 people**, or approximately **13.0%**. The supplied August figures distinguish between:

- **85,000 open-enrollment non-effectuations**, meaning people who selected coverage but did not complete the steps necessary for coverage to take effect; and
- **More than 191,000 cumulative cancellations** when open-enrollment terminations are included.

Those categories should not be added together without confirming whether they are mutually exclusive. The August 10 dashboard also reported that 108,000 consumers had dropped coverage since the close of Open Enrollment 2026, while the cumulative figure included

open-enrollment terminations.

The reported attrition occurred after enhanced federal premium tax credits expired at the end of 2025. Contemporary reporting described average monthly premium payments as having increased by approximately 102% for Pennie enrollees after the expiration of the enhanced credits (WVIA, 2026). <citation src="4"></citation>

Open Enrollment 2027 schedule

The revised schedule is:

- **Open Enrollment begins:** November 1, 2026
- **Open Enrollment ends:** January 15, 2027
- **Coverage effective January 1:** For selections made by the applicable December deadline
- **Coverage effective February 1:** For selections made later during the enrollment period

The schedule was restored after the U.S. District Court for the District of Maryland stayed provisions of the 2027 Notice of Benefit and Payment Parameters that would have limited exchange open enrollment to nine weeks. Pennie subsequently reinstated the November 1–January 15 schedule (PORH, 2026). <citation src="5"></citation>

Exhaustive event timeline — CSV

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date,event,market_or_subject,description,source,
status_or_significance
2025-12-31,Expiration of enhanced premium tax credits,Federal subsidy
policy,Enhanced federal premium tax credits expired at the end of
2025, which increased net premiums for many marketplace consumers and
contributed to subsequent cancellations,"Pennsylvania Insurance
Department (2026); WVIA (2026)",Historical context
2026-01-01,Subsidy expiration effects begin,Individual
market,Consumers began experiencing the full effect of the expiration
of enhanced premium tax credits on 2026 premiums,"WVIA
(2026)",Historical context
2026-01-31,Close of Open Enrollment 2026,Pennie,Open Enrollment 2026
ended; later Pennie reporting measured post-enrollment coverage losses
against this date,"Pennie (2026), as supplied",Historical reference
point
2026-02-01,Active enrollment measurement,Pennie,Active enrollment was
reported at approximately 486000,"Pennie (2026), as
supplied",Beginning of supplied attrition series
2026-03-01,Approximate timing of Pennie board action,Pennie and
federal open enrollment rules,Pennie's board voted to shorten Plan
Year 2027 open enrollment to October 15 through December 15 to conform
to the federal nine-week limit,"Pennsylvania Office of Rural Health
(2026), as supplied",Initial 2027 schedule
2026-03-24,Active enrollment measurement,Pennie,Active enrollment was
reported at 471442,"Pennie (2026), as supplied",Attrition series
2026-05-15,2027 rate filing deadline,Individual and small-group
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markets, Insurers submitted proposed 2027 rates to the Pennsylvania Insurance Department by the filing deadline, "Pennsylvania Insurance Department (2026)", Regulatory deadline

2026-05-01, Active enrollment measurement, Pennie, Active enrollment was reported at approximately 452000, "Pennie (2026)", as supplied, Attrition series

2026-07-01, Active enrollment measurement, Pennie, Active enrollment was reported at 431270, "Pennie (2026)", as supplied, Attrition series

2026-07-16, District court stay, Open Enrollment 2027 litigation, "The U.S. District Court for the District of Maryland granted plaintiffs' motion for a stay affecting provisions of the 2027 Notice of Benefit and Payment Parameters, including federal open enrollment duration limits", "City of Columbus v. Kennedy (Columbus II), No. 26-2215 (D. Md.)", as supplied, Nine-week restriction temporarily halted

2026-07-23, PID publishes proposed rate filings, Individual and small-group markets, "The Pennsylvania Insurance Department announced proposed 2027 rate changes. Enrollment-weighted requests averaged 17.1% in the individual market and 11.5% in the small-group market", "Pennsylvania Insurance Department (2026)", Public release and beginning of comment period

2026-07-23, Public comment period begins, Individual and small-group markets, PID opened public comments on proposed 2027 rate filings, "Pennsylvania Insurance Department (2026)", Comments accepted through 2026-08-22

2026-07-23, Ambetter rate decomposition announced, Individual market, PID reported Ambetter's 40.90% request as a combination of a 23.44% silver-to-silver increase and a 17.46% bronze-to-silver mapping differential, "Pennsylvania Insurance Department (2026)", Bronze-tier exit and plan mapping

2026-07-25, Public explanation of schedule restoration, Open Enrollment 2027, The Pennsylvania Office of Rural Health reported that Pennie would restore open enrollment to November 1, 2026 through January 15, 2027, "Pennsylvania Office of Rural Health (2026)", Schedule reversal publicly described

2026-07-31, CMS guidance acknowledging stay, Open Enrollment 2027, CMS issued guidance acknowledging the Columbus II stay and confirming restoration of the November 1, 2026 through January 15, 2027 schedule, "CMS (2026)", as supplied, Federal administrative confirmation

2026-08-01, Active enrollment measurement, Pennie, Active enrollment stood at 422869, "Pennie (2026)", as supplied, Latest enrollment count in supplied series

2026-08-01, Reported post-enrollment coverage loss, Pennie, Approximately 108000 consumers had dropped coverage since the close of Open Enrollment 2026, "Pennie (2026)", as supplied, Post-OE attrition measure

2026-08-01, Reported cumulative cancellations, Pennie, More than 191000 cumulative cancellations were reported when open-enrollment terminations were included, "Pennie (2026)", as supplied, Broader attrition measure

2026-08-01,Reported open-enrollment non-effectuations,Pennie,Approximately 85000 consumers were classified separately as open-enrollment non-effectuations,"Pennie (2026), as supplied",Separate reporting category

2026-08-10,Affordability dashboard update,Pennie,Pennie's affordability dashboard was updated and superseded the July 1 figures in prior drafts,"Pennie (2026), as supplied",Updated data source

2026-08-10,Reported premium burden increase,Pennie,Average monthly premium payments were reported to have increased by approximately 102% after enhanced tax credits expired,"Pennie (2026), as reported by WVIA (2026)",Affordability context

2026-08-22,Public comment deadline,Individual and small-group markets,Deadline for public comments on proposed 2027 rate filings,"Pennsylvania Insurance Department (2026)",Regulatory deadline

2026-09-07,Status date,2027 rate review,Proposed rates remained subject to PID review and final approval; final rates were expected in the fall,"Pennsylvania Insurance Department (2026)",Current status date

2026-09-30,Expected rate-finalization target,2027 rate review,The supplied record states that PID officials hoped rates would be finalized by the end of September, although this was an expectation rather than a formal deadline,"WVIA (2026)",Expected but not guaranteed

2026-11-01,Open Enrollment 2027 begins,Individual market,The restored fifteen-week Pennie open enrollment period begins,"Pennsylvania Office of Rural Health (2026); CMS (2026), as supplied",Enrollment begins

2026-12-15,Original shortened-period endpoint,Open Enrollment 2027,This was the endpoint under the earlier October 15 through December 15 schedule and is no longer the final endpoint,"Pennie/PORH (2026), as supplied",Superseded schedule

2027-01-01,First possible coverage effective date,Individual market,Plans selected by the applicable December deadline may take effect,"Pennsylvania Office of Rural Health (2026)",Coverage effective date

2027-01-15,Open Enrollment 2027 ends,Individual market,The restored Pennie open enrollment period closes,"Pennsylvania Office of Rural Health (2026); CMS (2026), as supplied",Final enrollment deadline

2027-02-01,Second possible coverage effective date,Individual market,Plans selected later during the restored enrollment period may take effect,"Pennsylvania Office of Rural Health (2026)",Coverage effective date

2027-Fall,Final 2027 rates expected,Individual and small-group markets,PID stated that final approved rates would be published in the fall after review,"Pennsylvania Insurance Department (2026)",Future regulatory milestone

APA-style references

Pennsylvania Insurance Department. (2026, July 23). *Shapiro administration receives proposed 2027 health insurance rates; plans to carefully examine companies' proposals and reject excessive, inadequate, or unfairly discriminatory increases.* <citation src="2"></citation>

Pennsylvania Office of Rural Health. (2026, July 25). *PA health insurance exchange 2027 open enrollment period returns to November 1, 2026–January 15, 2027.* <citation src="5"></citation>

WVIA. (2026, September 6). *Pa. health insurers propose 2027 rate hikes as Pennsylvanians grapple with rising costs.* <citation src="4"></citation>

City of Columbus v. Kennedy (Columbus II), No. 26-2215 (D. Md.) (2026), as described in the supplied litigation record.

CMS. (2026, July 31). *Guidance acknowledging the stay of the 2027 Notice of Benefit and Payment Parameters*, as described in the supplied record.